



My Courses (<https://learningcenter.hfsa.org/Users/Home.aspx>)
/ **2026 AHFTC Board Certification Review - OnDemand**
(<https://learningcenter.hfsa.org/Users/UserOnlineCourse.aspx?LearningActivityID=4f%2fOexzNsorKAF1lCFotvA%3d%3d>)
/ **2026 AHFTC Board Certification Question Bank**
(<https://learningcenter.hfsa.org/Users/UserOnlineCourse.aspx?LearningActivityID=YSpCYks4EUa7JCy700Rkkw%3d%3d>)
/ **Practice Exam - 1**

Correct Answer

Next Q

Practice Exam - 1

Question 81 of 117



A 68-year-old man with hypertension and diabetes presents with new-onset heart failure characterized by progressive dyspnea on exertion and abdominal bloating. He is admitted for decompensated heart failure and undergoes diuresis.

Transthoracic echocardiography demonstrates LVEF 35%, a dilated left ventricle, and moderate-to-severe mitral and tricuspid regurgitation. Coronary angiography reveals nonobstructive coronary artery disease, consistent with nonischemic cardiomyopathy.

He is started on Spironolactone 12.5 mg daily, Sacubitril/valsartan 24/26 mg twice daily, and Empagliflozin 10 mg daily, and is transferred to a tertiary center for valvular evaluation 3 days later.

After achieving euvolemia, repeat transthoracic echocardiography shows persistent at least moderate mitral regurgitation. Transesophageal echocardiography demonstrates severe functional mitral regurgitation anatomically suitable for transcatheter edge-to-edge repair, along with moderate-to-severe tricuspid regurgitation. Blood pressure is 126/82 mmHg and heart rate is 78 bpm.

Which of the following is the best next step in management?



A. Start metoprolol succinate



B. Proceed with surgical mitral and tricuspid valve repair

C. Proceed with mitralclip

D. Initiate evaluation for advanced therapies

Commentary

References

Testing Point: The Board-certified AHFTC specialist should recognize that GDMT should be optimized prior to intervention for functional mitral regurgitation.

Correct Answer: A. Start metoprolol succinate

Rationale: The patient is presenting with new onset heart failure with a new diagnosis of nonischemic cardiomyopathy as well as functional MR and TR. Prior to intervening on functional MR/TR, a trial of optimal GDMT is recommended as secondary MR can improve with improvement in LV dysfunction and geometry. The 2022 ACC/AHA HF guidelines give this a Class I recommendation indication. This patient is on low doses of angiotensin blocking drugs, MRA and full dose SGLT2 inhibitor and is not on beta blocker. Full medical therapy optimization with reassessment of regurgitation should occur prior to valve intervention. The patient's robust blood pressure and heart rate suggest more optimization is feasible.

Answers B & C: Until medication optimization occurs one should not proceed with transcatheter or surgical valve interventions for functional mitral and tricuspid regurgitation.

Answer D: The patient is newly diagnosed, as yet has not had a trial of medical therapy, and has robust blood pressure and heart rate, suggesting he does not have advanced or end stage disease, thus it is likely too early to evaluate for advanced therapies.

Powered by Oasis.



(<https://www.oasis-lms.com/>).